

D350

Psychiatric Mental Health Nurse Practitioner Clinical Internship III

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

A PMHNP student helps a busy family practice that serves many farmworker families start routine PHQ-9 screening for adults at annual visits. The clinic recently repainted its waiting room. Which level of prevention is this?

- A. Primary prevention
- B. Tertiary prevention
- C. Secondary prevention
- D. Environmental prevention through a calmer waiting room

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

A program provides medication support, skills training, and relapse-warning plans for adults recently discharged after a first psychotic episode. What level of prevention is this?

- A. Secondary prevention
- B. Tertiary prevention
- C. Primary prevention
- D. Not prevention, since the illness has already occurred

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A patient with social anxiety who works as a bank teller gains confidence after successfully ordering coffee, then asking a stranger for directions, then speaking up in a small meeting. His sister also encourages him often. What is the main source of his growing self-efficacy?

- A. Physiological states
- B. Verbal persuasion from his sister
- C. Vicarious experience
- D. Mastery experiences

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

In a recovery group, a woman decides she can manage her medications after watching a peer with the same diagnosis do it successfully. Which source of self-efficacy is this?

- A.** Verbal persuasion
- B.** Mastery experience
- C.** Vicarious experience
- D.** Emotional arousal

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A 35-year-old software tester with moderate depression can reasonably be treated with either CBT or an SSRI. She says she prefers not to take pills and has a flexible work schedule. Her insurance plan changed in January. What is the best approach?

- A.** Prescribe an SSRI because it is faster to start.
- B.** Decide based on which option the new insurance plan pays more for.
- C.** Discuss the benefits and risks of each option, confirm her preference, and choose CBT together with a plan to reassess.
- D.** Give her a list and let her decide alone.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A 40-year-old with schizophrenia, stable on medication, lives in supported housing and wants to return to part-time work at a bakery. His favorite color is green. Which plan best reflects recovery-oriented care?

- A.** Build the plan around his work goal with supported employment, while maintaining symptom management.
- B.** Delay work until all symptoms are completely gone.
- C.** Let the treatment team choose his goals.
- D.** Focus the plan only on medication adherence.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A patient with bipolar disorder has missed three appointments and run out of lithium twice. She explains the bus route to the clinic was cut and she cannot afford rideshares. She is a talented quilter. What is the best response?

- A. Address the transportation barrier: offer telehealth where appropriate, link to transport resources, and coordinate lab access.
- B. Switch to a different mood stabilizer.
- C. Document her as non-adherent and unmotivated.
- D. Suggest she sell quilts to pay for rides.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A county had 50,000 adults without a psychotic disorder at the start of last year, and 150 of them developed first-episode psychosis during the year. Another 600 residents were already living with a psychotic disorder at the start of the year. The county opened a new crisis center in March. What was the annual incidence?

- A. 15 per 1,000 adults
- B. 3 per 1,000 adults
- C. 333 per 1,000 adults
- D. 12 per 1,000 adults

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9. [TRANSFER — reversed stem / different variable]

A town of 80,000 residents has 2,400 people currently living with major depression; 200 of them were diagnosed this year. What is the point prevalence?

- A. 8.3%
- B. 0.25%
- C. 3.25%
- D. 3%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A patient with a history of assault needs a physical exam at intake. She seems tense and has a service dog named Max. Which approach best reflects trauma-informed care?

- A.** Ask her to describe the assault in detail first.
- B.** Require the service dog to leave the room.
- C.** Complete the exam quickly without talking to limit her distress.
- D.** Explain each step before doing it, ask permission, and offer choices such as a chaperone or pausing.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

At a community health fair held in a school gym, 1,000 adults take a depression screen. Interviews show 120 have major depression; 102 of them screened positive. Of the 880 without depression, 88 also screened positive. The fair also offered free flu shots. What is the sensitivity of the screen?

- A.** 85%
- B.** 53.7%
- C.** 90%
- D.** 10.2%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

Among 200 people who do not have an anxiety disorder, a screening tool is negative for 170. What is the specificity?

- A.** 100%, since most tested negative
- B.** 85%
- C.** 15%
- D.** It cannot be calculated without the number who have the disorder.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer C Concept 1

A PMHNP student helps a busy family practice ~~that serves many farmworker families~~ start routine PHQ-9 screening for adults at annual visits. ~~The clinic recently repainted its waiting room.~~ Which level of prevention is this?

Target word: prevention level **Decides:** Screening finds existing but undiagnosed illness.

A	Adjacent category	Primary prevention stops onset, e.g., resilience training.
B	Inverted operation	Tertiary targets people already diagnosed.
C	Correct	Screening detects disease early.
D	Camouflage	Decor is not the intervention.

2. Answer B Concept 1 • transfer

A program provides medication support, skills training, and relapse-warning plans for adults recently discharged after a first psychotic episode. What level of prevention is this?

Target word: reverse: identify tertiary **Decides:** Already diagnosed; reduce relapse and disability.

A	Confusion set	Secondary is early detection.
B	Correct	Limits disability and relapse in established illness.
C	Inverted operation	Primary targets people without the disorder.
D	Absolute language	Tertiary prevention applies after diagnosis.

3. Answer D Concept 2

A patient with social anxiety ~~who works as a bank teller~~ gains confidence after successfully ordering coffee, then asking a stranger for directions, then speaking up in a small meeting. His sister also encourages him often. What is the main source of his growing self-efficacy?

Target word: strongest source **Decides:** He succeeded at the tasks himself.

A	Adjacent category	Refers to how bodily arousal is interpreted.
B	Camouflage	Encouragement helps but is weaker and not the main driver here.
C	Confusion set	Vicarious means watching others succeed.
D	Correct	Personal success is the strongest source of self-efficacy.

4. Answer C Concept 2 • transfer

In a recovery group, a woman decides she can manage her medications after watching a peer with the same diagnosis do it successfully. Which source of self-efficacy is this?

Target word: reverse: vicarious **Decides:** She watched a similar other succeed.

A	Confusion set	No one told her she could do it.
B	Inverted operation	She has not done it herself yet.
C	Correct	Learning from a similar model's success.
D	Adjacent category	Not about interpreting bodily states.

5. Answer C Concept 3

A 35-year-old software tester with moderate depression can reasonably be treated with either CBT or an SSRI. She says she prefers not to take pills and has a flexible work schedule. ~~Her insurance plan changed in January.~~ What is the best approach?

Target word: shared decision **Decides:** Equivalent options and a stated preference.

A	Inverted operation	Overrides her preference without need.
B	Camouflage	Coverage can matter but is not the core of SDM here.
C	Correct	SDM integrates evidence and her values.
D	Dropped element	Leaves out clinician guidance.

6. Answer A Concept 4

A 40-year-old with schizophrenia, stable on medication, lives in supported housing and wants to return to part-time work at a bakery. ~~His favorite color is green.~~ Which plan best reflects recovery-oriented care?

Target word: recovery-oriented plan **Decides:** Person-directed goals across life domains.

A	Correct	Recovery centers the person's own purpose and community goals.
B	Absolute language	Recovery does not require symptom elimination.
C	Inverted operation	Recovery goals are person-directed.
D	Dropped element	Leaves out purpose, home, and community.

7. Answer A Concept 5

A patient with bipolar disorder has missed three appointments and run out of lithium twice. She explains the bus route to the clinic was cut and she cannot afford rideshares. ~~She is a talented quilter.~~ What is the best response?

Target word: address SDOH **Decides:** Transportation barrier.

A	Correct	Solves the actual cause of missed care.
B	Adjacent category	Changing drugs does not fix access.
C	Inverted operation	Mislabels a practical barrier.
D	Camouflage	Uses an irrelevant detail instead of linking resources.

8. Answer B Concept 6

A county had 50,000 adults without a psychotic disorder at the start of last year, and 150 of them developed first-episode psychosis during the year. Another 600 residents were already living with a psychotic disorder at the start of the year. ~~The county opened a new crisis center in March.~~ What was the annual incidence?

Target word: incidence **Decides:** New cases only / population at risk.

A	Confusion set	Adds new and existing cases (750), which is closer to period prevalence.
B	Correct	$150 / 50,000 \text{ at risk} = 0.003 = 3 \text{ per } 1,000.$
C	Inverted operation	Divides 50,000 by 150.
D	Wrong stem number	Uses the 600 existing cases.

9. Answer D Concept 6 · transfer

A town of 80,000 residents has 2,400 people currently living with major depression; 200 of them were diagnosed this year. What is the point prevalence?

Target word: solve for prevalence **Decides:** All current cases / total population.

A	Adjacent category	200 / 2,400 is the share of cases that are new.
B	Confusion set	200 / 80,000 is the new-case rate.
C	Inverted operation	Adds new cases that are already included in the 2,400.
D	Correct	2,400 / 80,000 = 3%.

10. Answer D Concept 7

A patient with a history of assault needs a physical exam at intake. She seems tense ~~and has a service dog named Max~~. Which approach best reflects trauma-informed care?

Target word: trauma-informed approach **Decides:** Empowerment, voice, choice, transparency.

A	Inverted operation	Pressing for trauma details can retraumatize.
B	Camouflage	The dog may support her safety; it is not the issue.
C	Dropped element	Removes transparency and choice.
D	Correct	Builds safety, trust, and choice.

11. Answer A Concept 8

At a community health fair ~~held in a school gym~~, 1,000 adults take a depression screen. Interviews show 120 have major depression; 102 of them screened positive. Of the 880 without depression, 88 also screened positive. ~~The fair also offered free flu shots~~. What is the sensitivity of the screen?

Target word: sensitivity **Decides:** True positives / all with the condition.

A	Correct	102 / 120 = 85%.
B	Adjacent category	102 / 190 is the positive predictive value.
C	Confusion set	792 / 880 is the specificity.
D	Wrong stem number	102 / 1,000 divides by everyone screened.

12. Answer B Concept 8 · transfer

Among 200 people who do not have an anxiety disorder, a screening tool is negative for 170. What is the specificity?

Target word: specificity **Decides:** True negatives / all without the condition.

A	Absolute language	30 false positives lower specificity.
B	Correct	170 / 200 = 85%.
C	Inverted operation	30 / 200 is the false-positive rate.
D	Dropped element	Specificity uses only those without the condition.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owled.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	6	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	10	
Absolute language	Always, never, guarantee — where judgment applies.	3	
Confusion set	Every option is real; the test is exact role-matching.	6	
Camouflage	Answers a stem fact that was never the question.	5	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Levels of prevention	Primary prevention stops a disorder before it starts; secondary detects it early (screening); tertiary reduces disability and relapse in established illness.	Calling screening primary prevention.	Whether the target group is healthy, undiagnosed, or already diagnosed.
2	Sources of self-efficacy	Bandura's sources are mastery experiences (strongest), vicarious experiences (watching similar others succeed), verbal persuasion, and physiological/emotional states.	Treating encouragement as the strongest source.	How a patient's confidence grew.
3	Shared decision making	When options have similar evidence, the clinician explains benefits and risks, elicits the patient's values and preferences, and decides together.	Choosing for the patient or simply handing over a list.	Two reasonable options and a patient with a preference.
4	Recovery-oriented care	Recovery is person-directed and holistic, built on health, home, purpose, and community (SAMHSA), with goals the person defines.	Defining recovery as symptom elimination set by the clinician.	A plan built around a person's own life goals.
5	Social determinants of health	Barriers such as transportation, housing, food, and income strongly affect outcomes; screen for them and link to resources as part of the plan.	Labeling missed care as non-adherence or lack of motivation.	A practical barrier behind missed visits or medications.
6	Incidence vs. prevalence	Incidence = new cases in a period / population at risk; prevalence = all existing cases at a point or period / total population.	Counting existing cases in incidence or new cases only in prevalence.	Both new and existing case counts appear.
7	Trauma-informed care	SAMHSA's principles are safety; trustworthiness and transparency; peer support; collaboration and mutuality; empowerment, voice, and choice; and attention to cultural, historical, and gender issues.	Pressuring patients to retell trauma or removing their choices for efficiency.	A patient with trauma history facing a procedure or decision.
8	Screening test accuracy	Sensitivity = true positives / all with the condition; specificity = true negatives / all without it; positive predictive value = true positives / all positives.	Using the wrong denominator.	A two-by-two table of screening results.

Course scope (public description): Through precepted clinical experiences, the learner will develop competencies needed to provide comprehensive and holistic mental health care to individuals, families, and

communities across the lifespan. In the precepted clinical setting, the learner will combine competencies developed in preparatory advanced practice coursework to deliver consumer- centered mental health care. Therefore, the learner will conduct advanced mental health assessments and utilize the competencies of advanced pathophysiology, psychopharmacology, psychotherapy and health promotion for individuals and populations across the life span to build self-efficacy in individuals and groups, to determine correct diagnoses, and to establish mental health plans of care that include consumer and population preferences. All MSN Core Courses, NP Core courses, and PMHNP Specialty courses must be completed before taking this course.